



EquipSeva

BIOMEDICAL SERVICE + TEAM OPERATIONS

PLANNING OVERVIEW

One service network. Clear responsibilities.

A connected Android experience for hospitals, biomedical engineers and engineering businesses.

Continue the existing app and backend. Preserve working accounts, records, the original logo and the lime/ink design while delivering new capabilities in bounded stages.

01 / SERVICE

Service with accountability

Hospitals request and confirm work; verified engineers perform authorised repairs.

02 / WORKSPACE

Teams with boundaries

A paid organisation manages its own staff, assignments and business operations.

03 / ASSURANCE

Trust backed by evidence

Server checks, payment records and independent review govern live access.

A plan for delivery, not a release claim.

98 existing screen declarations + 30 proposed surfaces are an inventory, not implementation or completion percentages. Current app acceptance still has open test, visual, security and provider/device gates.

Four people. Three public choices.

Registration declares intent. Verified identity and server-authorised context decide what a person can do.

PUBLIC CHOICE 01

Biomedical engineer

Personal practice or assigned team work: quote, prepare, repair, record evidence and track earnings.

PUBLIC CHOICE 02

Hospital administrator

Claim or join an authorised site, request service, approve terms and confirm or dispute completion.

PUBLIC CHOICE 03

Engineering team

An organisation or sole operator manages consenting staff, dispatch, reports and subscription.

PRIVATE / NO SIGNUP CARD

Platform owner

Privately provisioned operations: verification, disputes, incidents and auditable support. MFA required.

ONE SHARED SERVICE LIFECYCLE

1

Request + quote

Hospital defines the need. Eligible engineers or teams quote.

2

Agree + assign

Accepted scope, service parties, engineer and beneficiary are recorded.

3

Visit + report

Attendance challenge, equipment evidence and service report.

4

Confirm + settle

Independent hospital confirmation or dispute; verified payment and receipt.

TEAM NAVIGATION

Overview / Jobs / Team / Organisation

Access is earned in separate checks.

Payment unlocks eligible software features within an authorised organisation. It does not create trust or platform power.

Identity + Workspace authority + Capability

Then apply resource/site ownership, lifecycle state and any required entitlement. Every private read, write, attachment, export and realtime channel follows the same server boundary.

Personal engineer workspace

Match the signed-in user to the immutable personal owner. No company membership or team subscription is required.

Organisation / hospital scope

Current membership and resource authority are required. Hospital managers and staff need explicit site grants; empty scope denies access.

Explore safely with a synthetic demo

Guided, resettable, offline fictional data. No shared production account, real payment, invitation or message. Demo repositories and navigation cannot call live mutation APIs; demo work is discarded on exit.

MONTHLY TEAM SUBSCRIPTION / PLANNED

One organisation plan with an explicit seat limit. Price, seats, tax treatment and refund/cancellation terms must be finalised before live charges.

Pending / Active / Grace / Restricted / Suspended

Server/provider state governs access. Cancellation keeps the paid-through period; grace must be provider-confirmed. Lapse blocks new paid commitments while preserving permitted existing work, receipts and disputes. Identity or security denial takes priority.

Two products, two ledgers: digital SaaS and physical repair/onsite AMC. Launch design uses Play Billing for in-app team subscriptions; any alternative program requires separate eligibility and integration review. [1-3]

District discovery. Evidence-led visits.

The target experience has no maps or GPS-dependent normal workflow. Server migration must happen before GPS removal.

India > State or Union Territory > District

Use reviewed LGD codes, catalog version and explicit alias history. Names are display labels. Preserve historical site, address and district snapshots. [4]

Match by declared coverage

Engineer: base district + selected service districts + equipment skills + availability.
Hospitals retain a district for each site.

Keep private travel details

Share the service address, landmark and contact only with authorised participants at the permitted booking state.

MAP-FREE ATTENDANCE CONTRACT / PROPOSED

01

Issue a bounded challenge

Server-issued, short-lived, single-use; bound to job, engineer and hospital participant.

02

Capture service evidence

Before/after equipment evidence and a durable service report with explicit status.

03

Obtain independent acceptance

Hospital countersign or dispute; duplicate-evidence and collusion review remain.

Missing representative or no network?

Keep a documented pending/manual-review path and emergency support. A forwarded code is not independent proof of physical presence. Test offline evidence durability before promising it; never fabricate check-in or zero coordinates.

Protect the boundaries that matter.

Layer client protections around authoritative server controls. No claim that the app is unhackable or uncloneable.

01 Account + workspace isolation

Capture the originating user, login generation and workspace. Validate inside the real storage or server mutation so delayed logout, uploads and callbacks cannot affect another context.

02 Money + contracts

Verify purchases and webhooks on the server; handle replay, reordered events and uncertain outcomes. Preserve agreed job parties and beneficiary revisions; a screenshot is never proof of payment.

03 Claims + evidence

Review engineer qualifications, hospital authority and team ownership separately. Detect copied evidence, collusion and payout redirects with bounded review and a human appeal path.

04 Privileged operations

Require MFA, least privilege, fresh authentication and an audit reason. No secret viewing, silent impersonation or self-approved exceptional money release. Founder power is constrained too.

Offline access is bounded, not unlimited.

The plan proposes a maximum 24-hour server-issued lease for minimal routine cached reads. No fresh offline billing, admin, assignment or financial authority. Reconnect revalidates; revocation denies new online operations.

Previously viewed or exported copies cannot be recalled. Collect equipment-service data, not patient records. Retention, privacy and provider/legal review remain launch dependencies. [5-7]

Staged delivery. Evidence gates.

P1 is the first live-behaviour milestone. Design preparation can proceed independently within safe boundaries.

- P0**

Plan + page mapping
 Product boundaries, source reconciliation and delivery ledger.
- P1**

Ownership + recovery
 Repair cleanup and session/navigation recovery before more workspaces.
- P2**

Tenants + regions
 Server permissions, site scope, canonical districts and safe migration.
- P3**

Entry + demo
 Three-choice registration, authorised joins and isolated fake-data demo.
- P4**

SaaS billing
 Verified entitlement, seats, purchase recovery and cancellation.
- P5**

Service journeys
 Hospital/engineer jobs, district discovery and map-free attendance.
- P6**

Team operations
 Invitations, dispatch, reporting, delegated admin and owner transfer.
- P7**

Remaining pages
 Profile, chat, AMC, platform operations, locales and accessibility.
- P8**

Pilot + release gates
 Real provider/device evidence, signed release and operating readiness.

Current application candidate is still blocked.

3,675 recorded unit tests; 3 cleanup failures.

116 changed screenshot cases; 2 previously sampled.

Recorded lint, debug and unsigned release assembly passed. These are prior results at candidate ccda4e4a, not rerun for this plan. Payment, integrity, recovery, full visual and device/provider acceptance remain open.

What must be true before launch.

A reviewed plan authorises staged work. Each delivered scope earns its own acceptance evidence.

Permission + money gates

Direct foreign-tenant, personal-owner and site-scope denials; replay and race tests; provider-verified billing; no account/workspace leakage.

Usability + human acceptance

Light/dark, EN/HI/TE, narrow screens, 2x text, TalkBack, Back/keyboard and recovery. Real tasks with hospitals, engineers, team administrators and platform operators.

Commercial + operating readiness

Finalize price, seats, terms, taxes/refunds, provider configuration and support ownership. Validate load profile, backup restore, alerts, incident response and privacy/retention.

Independent critic + QA: each at least 9.5 / 10

No unresolved critical/high blocker or missing mandatory evidence. Planning scores apply only to the plan; test, integration, device and human evidence are recorded separately.

READ THE GOVERNING PLAN AND DETAILED CONTRACTS

[Master product plan](#) / [Pages and workflows](#) / [Architecture](#)
[Security, payments and abuse](#) / [Delivery ledger](#)

PRIMARY REFERENCES / POLICY RECHECK REQUIRED AT LAUNCH

- [1] [Google Play payments policy](#)
- [2] [India alternative billing program](#)
- [3] [Play purchase-security guidance](#)
- [4] [Government of India Local Government Directory](#)
- [5] [Google Play Integrity overview](#)
- [6] [RBI payment-aggregator direction](#)
- [7] [MeitY DPDP Rules Gazette](#)